REPORT OF THE PORTFOLIO COMMITTEE ON HEALTH AND CHILD CARE

ON

PREVENTION AND MANAGEMENT OF COMMUNICABLE DISEASES (NCDs) INCLUDING CANCER

SECOND SESSION – TENTH PARLIAMENT

Presented to Parliament on --- 2024

S.C., 2024

ORDER OF APPOINTMENT FOR PORTFOLIO COMMITTEE

ORDERED IN TERMS OF STANDING ORDER NO.17:

At the commencement of every session, there shall be as many Committees to be designated according to government policy as the Standing Rules and Orders Committee may deem fit. Each select committee shall be known by the portfolio determined for it by the Standing Rules and Orders Committee.

TERMS OF REFERENCE OF PORTFOLIO COMMITTEES

STANDING ORDER NO. 20

Subject to these Standing Orders, a Portfolio Committee shall-

1. examine expenditure administration and policy of government departments and other matters falling under their jurisdictions as Parliament may, by resolution determine.
2. consider and deal with all bills other than a Constitutional Bill, and statutory instruments or other matters which are referred to it by or under resolution of the House or by the Speaker;
3. consider or deal with an appropriation or money bill or any aspect of an appropriation or money bill referred to it by these Standing Orders or by or under a resolution of this House;
4. monitor, investigate, inquire into and make recommendations relating to any aspect of the legislative programme, budget, policy or any other matter it may consider relevant to the government department falling within the category of affairs assigned to it, and may for that purpose consult and liaise with such department; and
5. consider or deal with all international treaties, conventions and agreements relevant to it, which are from time to time negotiated, entered into or agreed upon

SPEAKER'S ANNOUNCEMENT

On Tuesday, 12 October, 2023, the Speaker announced that the following Members shall serve on the Portfolio Committee on Health and Child care;

Hon. Chakukura A.
Hon. Bajila D.
Hon. Dumbarimwe T.
Hon. Hlatwayo C.
Hon. Kapoikulu S.
Hon. Matsunga S.
Hon. Makombe Josiah
Hon. Mahlangu S.
Hon. Molokele D.
Hon. Muchemwa W.
Hon. Mukuhlani T.
Hon. Muwodzeri T.
Hon. Mutandi G.
Hon. Ndudzo P.
Hon. Nhari V.
Hon. Nyamupinga Karimatsenga B.
Hon. Ndlovu Nomathemba
Hon. Shongedza E.
Hon. Pinduka T.
Hon. Tobaiwa J.
Hon. Zhou P.
Hon. Ziyambi S.

Hon. Dr. Makombe Josiah to be the Chairperson

Introduction

Zimbabwe has been making significant efforts to strengthen cancer management, particularly in the context of Non-Communicable Diseases (NCDs). However, the burden remains high and this has been exacerbated by the increased incidence of non-communicable diseases (NCDs) which accounted for 36% of deaths in 2019[1]. Recognising the urgency of this matter, the Portfolio Committee on Health and Child Care undertook fact-finding visits to various hospitals from the 13 to 16 May 2024. This visit aimed to evaluate the state of cancer treatment services and the management of NCDs in selected hospitals across different regions. The Committee visited six hospitals: Mpilo Central Hospital, Parirenyatwa Group of Hospitals, Gweru General Hospital, Makumbe District Hospital, Karanda Hospital and Bindura General Hospital. These hospitals play a crucial role in providing healthcare services to large catchment areas, often serving as referral centers for surrounding districts. To that end, the report presents the findings from these visits, highlighting critical areas such as health infrastructure, human resources for health, and health financing. The report aims to provide a detailed overview of the current state of cancer and NCD management in these hospitals, identify the key challenges faced, and offer recommendations for improvements to enhance healthcare delivery and outcomes for patients across the country.

2. Objectives

The primary objectives of the fact-finding visit were:

1. To assess the measures in place to effectively offer cancer treatment and NCD management services
2. To identify the challenges faced by these hospitals in providing adequate care for cancer and NCD patients.
3. To proffer recommendations aimed at addressing the challenges observed.

3. Methodology

The Committee's inquiry commenced with an oral evidence session with the Ministry of Health and Child Care. This session provided an overview of the national efforts and policies aimed at addressing NCDs and cancer treatment. Following this session, the Committee conducted visits to

[1] https://www.afro.who.int/news/strengthening-management-childhood-cancer-zimbabwe.

the six selected hospitals from 13-16 May 2024. During these visits, the Committee engaged with hospital administrators and medical staff, to gather first-hand information on the conditions and challenges faced.

4. Committee's Findings

4.1 Treattment and Management of cancer and NCDs

4.1.1 During oral evidence from the Ministry, it was highlighted that Zimbabwe had prioritized the management of NCDs as outlined National Health Strategy (2012-2025). The plan focused on strengthening the treatment and management of cardiovascular diseases, cancers, chronic respiratory diseases and diabetes. It was further mentioned that NCDs accounted for 36% of deaths in Zimbabwe, with cardiovascular diseases contributing 13%, cancers 9%, diabetes 5%, chronic respiratory diseases 2%, and other NCDs 7%. The mortality of these NCDs was worsened by major risk factors such as tobacco use, physical inactivity, unhealthy diets, and harmful alcohol use. These risk factors often led to conditions such as high blood pressure, obesity, hyperglycemia and hyperlipidemia. Furthermore, it was expressed that poor health seeking behaviours particularly among men, had led many patients to seek medical treatment only at advanced stages of their diseases, particularly for cancer, where early detection significantly improved treatment outcomes.

4.1.2 However, in response, it was highlighted that the Government had focused on early detection, screening, treatment, and palliative care, with cancer prevention strategies addressing infectious agents, reducing tobacco and alcohol use, promoting healthy diets, and utilizing vaccinations against Human Papilloma Virus (HPV) and Hepatitis B Viris (HBV). Moreover, radiotherapy and oncology services were available at Parirenyatwa and Mpilo Hospitals, but the diagnosis of cancers was often delayed, with 80-90% of patients presenting at advanced stages. Additionally, this untenable situation was aggravated by the critical shortage of specialists, including pathologists, radiologists, and oncologists, and advanced imaging facilities like Computed Tomography (CT) and Magnetic Resonance Imaging (MRI) which were largely unavailable in public hospitals.

4.2 Health Infrastructure

3.2.1 The Committee toured the health facilities and were alerted to the various challenges that had militated against their capacity to efficiently and effectively provide quality cancer and NCD treatment and management services. The Committee noted that at both Mpilo and Parirenyatwa Central Hospitals, critical radiotherapy machines had been non-functional since 2021. At Parirenyatwa, one of the two brachytherapy machines was down due to the exhaustion of Cobalt 60 radioactive sources, needs to replenished. Additionally, the linear accelerators, essential for targeted cancer treatments, had been non-functional for the past two years, severely impacting the hospital's ability to provide comprehensive cancer care. At Mpilo Central Hospital, the situation was similarly dire, with only 7 out of 14 renal treatment machines operational, reducing the hospital's capacity to provide necessary dialysis treatments.

3.2.2 The mammogram machine at Mpilo, which was donated by the National AIDS Council (NAC) to aid in breast cancer screening, was out of service, thereby halting vital early detection processes for breast cancer, a major concern given the high prevalence of late-stage cancer diagnoses. Further to that, the mobile X-ray unit at Mpilo was also down, causing significant delays in surgical procedures for children, who require timely diagnostic imaging for accurate and effective treatment. More so, although the laboratory services were fairly equipped with essential diagnostic tools, they suffered from an erratic supply and availability of key reagents. This inconsistency disrupted routine diagnostic testing and delayed critical medical investigations, undermining the overall efficiency of the hospital's healthcare delivery system. Consequently, the combination of outdated and malfunctioning equipment, coupled with supply chain issues, paints a challenging picture for the hospitals' ability to deliver timely and effective patient care.

3.2.3 However, Government must be commended for the on-going efforts to recapacitate health facilities, which are crucial steps towards improving healthcare delivery in the country. Notably, the recent procurement of two MRI machines for Mpilo and Parirenyatwa Central Hospitals represented a significant advancement in diagnostic capabilities. These state-of-the-art machines enhanced the hospitals' ability to conduct detailed imaging studies, leading to more accurate diagnoses and better treatment planning for a range of conditions,

including cancer and other non-communicable diseases. This investment, indeed, reflects the Government's commitment to addressing the critical needs of the healthcare system and demonstrates a proactive approach to modernizing medical infrastructure, which was essential for delivering high-quality healthcare services to the population.

4.4 Human Resources for Health

4.1.1 During the tour, the Committee was made aware of high level of skills flight to green pastures in the hospitals. It was highlighted that most health facilities were operating below optimal levels due to significant shortages of key personnel such as radiographers, medical physicists, and nuclear medicine physicians. This shortage of skilled staff was exacerbated by the increased workload and the rapid advancements in medical fields, which necessitated continuous upskilling of health professionals. However, limited funding restricted opportunities for professional development, leaving many healthcare workers without the necessary training to keep pace with new technologies and treatment protocols. Additionally, the shortage of skilled personnel was compounded by obsolete medical infrastructure, further hampering the effective delivery of healthcare services. This combination of understaffing, lack of training opportunities, and outdated equipment created a challenging environment for healthcare providers and adversely affected patient care outcomes.

4.5 Health Financing

4.5.1 The visits revealed that most hospitals were underfunded and this had negatively affected service delivery. It was pointed out that health financing in Zimbabwe had been a persistent challenge, with public healthcare funding relying heavily on government allocations and external donor support. The limited national budget had resulted in insufficient funding for hospitals, medical supplies, infrastructure maintenance, and staff salaries. Moreover, the hospitals bemoaned the inconsistent disbursements and the large discrepancies between the allocated and the disbursed funds. This situation had led these health facilities to struggle to offer and maintain essential services and/or invest in advanced medical technologies and comprehensive training programs for healthcare workers.

4.5.2 Additionally, in some instances, such as the provision of renal services, the Government often provided renal kits to support patients undergoing dialysis. However, patients were still expected to purchase other sundry materials, such as catheters, which significantly increased their out-of-pocket spending on health. This additional financial burden was particularly challenging for many patients, who already faced high medical costs due to their conditions. The requirement to buy supplementary medical supplies highlighted a gap in the healthcare system's ability to fully support patients, further exacerbating the financial strain on individuals and families dealing with chronic health issues.

4.5.3 However, Government was commended for its to reduce the burden of accessing health services. Notably, in the 2024 national budget, there was a significant increase in per capita spending on health to US$71.8, moving closer to the National Development Strategy 1 (NDS1) target of $84. This increase represented a substantial commitment to improving healthcare infrastructure, services, and accessibility for all citizens. Additionally, the introduction of the sin tax on sugar, which was specifically ring-fenced to procure cancer treatment equipment, demonstrated a strategic approach to health financing.

5.0 Committee's Observations

The Committee made the following observations:

1. NCDs accounted for 36% of deaths in Zimbabwe, with major risk factors being tobacco use, physical inactivity, unhealthy diets, and harmful alcohol use.
2. Poor health-seeking behaviors, especially among men, resulted in late-stage disease presentations, notably for cancer.
3. Radiotherapy and oncology services were available at certain hospitals, but diagnosis was often delayed due to a shortage of specialists and advanced imaging facilities.
4. While some services were offered, patients were often required to purchase other sundry materials such as catheters which significantly increased out-of-pocket spending on health.
5. Hospital visits revealed critical infrastructure challenges like non-functional radiotherapy machines and limited renal treatment capacity.
6. Most health facilities could not operate optimally due to the high rates of attrition of key medical personnel such as theatre nurses.

7. The user fee policy had affected the capacity of health facilities to generate revenue to support their operations.

6.0 Recommendations

1. The Ministry of Health and Child Care should establish binding maintenance contracts with health equipment suppliers. These contracts should include comprehensive training programs for local personnel, covering both the use and ongoing maintenance of the equipment.
2. There is need for the Ministry of Health and Child Care to formulate a policy to ensure that training schools retain some of the health personnel by June 2025.
3. There is need for the Ministry of Health and Child Care to develop a roadmap for the decentralization of cancer treatment facilities to all district hospitals by June 2025. This includes procurement of mobile clinics and capacitation of primary health care facilities to provide these services.
4. There is need for a deliberate strategy to train, recruit, and retain key medical personnel, especially oncologists and theatre nurses. This strategy should focus on providing competitive salaries, opportunities for professional development, and supportive working conditions by June 2025.
5. There is need for the Ministry of Health and Child Care in conjunction with relevant Ministries, Departments, agencies and development partners to conduct awareness campaigns on cancer and NCDs treatment and management by December 2025.
6. There is need for the Ministry of Health and Child Care in conjunction with the Ministry of Higher and Tertiary Education, Innovation, Science and Technology Development to conduct research and development into traditional medicines and their efficacy in cancer and NCD treatment and management by December 2025.
7. There is need for the Ministry of Health and Child Care to establish palliative care centres in all hospitals by December 2025.

7.0 Conclusion

The fact-finding visit highlighted critical issues in the management and treatment of cancer and NCDs in Zimbabwean hospitals. The limitations in infrastructure, human resources, and financing severely affect the quality of care provided to patients. Addressing these challenges requires a coordinated effort by the government, healthcare institutions, and stakeholders to improve healthcare delivery and outcomes for patients with cancer and other NCDs.